

# CITY COMPREHENSIVE CANCER CENTER

Department of Gastrointestinal Oncology

Date of Evaluation: August 8, 2026

**Patient Name:** Robert Kensington

**DOB / Age:** 21/08/1958 | 67 y.o.

**Sex:** Male

**MRN:** #554-90-C

**Attending Physician:** Dr. J. Lin, MD, FACP

## CHIEF COMPLAINT (CC)

Post-colonoscopy follow-up and staging discussion.

## HISTORY OF PRESENT ILLNESS (HPI)

Mr. Kensington is a 67-year-old male who initially presented to his primary care physician two months ago reporting a 3-month history of intermittent hematochezia (bright red blood mixed with stool), narrowing of stool caliber, and an unintentional weight loss of 5 kg. A diagnostic colonoscopy was performed on July 15, 2026, which revealed a 4.5 cm partially obstructing, ulcerated, and friable mass in the sigmoid colon. Biopsies confirmed moderately differentiated invasive adenocarcinoma. Staging CT of the chest, abdomen, and pelvis (July 28, 2026) revealed local pericolic lymph node enlargement but no evidence of distant hepatic or pulmonary metastasis.

## PAST MEDICAL HISTORY

Type 2 Diabetes Mellitus (well-controlled on Metformin). Hypertension. No family history of colorectal cancer or inflammatory bowel disease.

## PHYSICAL EXAMINATION

Vitals: BP 130/80, HR 76, Temp 37.1C, Wt 78 kg.

General: Well-appearing, NAD.

Abdomen: Soft, non-distended, mild tenderness to deep palpation in the Left Lower Quadrant (LLQ). No palpable masses or hepatosplenomegaly. Normoactive bowel sounds.

DRE: Normal sphincter tone, no rectal masses palpable, trace fecal occult blood positive.

## ASSESSMENT & PATHOLOGY

- Diagnosis:** Primary Adenocarcinoma of the Sigmoid Colon.
- Clinical Staging:** cT3, cN1, cM0 (Clinical Stage IIb).

- **Mismatch Repair (MMR) Status:** Proficient (pMMR) / MSS. KRAS: Wild-type.

## TREATMENT PLAN (TUMOR BOARD RECOMMENDATIONS)

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1. **Surgical Intervention:** Patient is referred to Colorectal Surgery (Dr. H. Vance) for a scheduled laparoscopic sigmoid colectomy with en bloc regional lymphadenectomy.
2. **Adjuvant Chemotherapy:** Given the clinical node-positive status, adjuvant chemotherapy is highly recommended post-resection. We discussed the FOLFOX regimen (Fluorouracil, Leucovorin, and Oxaliplatin) for a duration of 3 to 6 months depending on final pathologic staging. Patient understands the risks, including peripheral neuropathy associated with Oxaliplatin.
3. **Pre-operative Optimization:** Refer to anesthesia for pre-op clearance. Continue current anti-diabetic and anti-hypertensive medications.
4. **Follow-up:** Surgery consultation scheduled for next Tuesday. Oncology follow-up to occur 3 weeks post-operation to review surgical pathology and initiate systemic therapy.

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Dr. J. Lin, MD, FACP

Electronic Signature Validated